CLINICAL · APPROACH TO

Approach to a lump in the throat

Globus pharyngeus is the persistent feeling of a lump, tightness or something stuck in the throat, with no difficulty swallowing food. It is common, accounts for a large share of new referrals to ENT clinics, and is benign; but patients are often frightened that it is cancer, and a minority of cancers present with throat symptoms that can be mistaken for globus. The approach is to recognise the typical pattern, look for the features that make it atypical, examine the throat thoroughly, and then explain and treat with confidence rather than investigate endlessly.

AT A GLANCE

- Globus: midline, comes and goes, eating relieves it.
- Red flags: true dysphagia, pain, otalgia, lateralised, voice change, weight loss, neck lump.
- Flexible nasendoscopy in clinic; no routine barium.
- Reassurance, reflux measures, speech therapy.

1 CHAPTER ONE Typical globus

The classic history.

1.1 The pattern

Typical globus is felt in the midline, usually around the level of the cricoid or the thyroid notch. It is present **between meals** and when swallowing saliva, and often **improves when eating or drinking**. It is not painful and does not cause true dysphagia, odynophagia, weight loss or regurgitation. It fluctuates, worsens with stress, tiredness and throat clearing, and has often been present for months. It is most common in middle age and slightly more common in women.

1.2 What lies behind it

The cause is usually multifactorial. Suggested contributors include **laryngopharyngeal reflux**, raised cricopharyngeal tone, postnasal drip, throat clearing, muscle tension around the larynx, stress and anxiety, and heightened

sensitivity of the pharynx. Thyroid swelling, cervical osteophytes, a long uvula and tongue-base lymphoid hypertrophy are occasionally relevant.

FEATURES THAT ARE NOT GLOBUS

- **Pain, odynophagia or referred otalgia.**
- **True dysphagia,** food sticking, regurgitation or weight loss.
- **Unilateral or lateralised symptoms,** or a sensation that is progressive.
- **Hoarseness,** haemoptysis or a neck lump.
- **A smoker or heavy drinker over 50.**

Any of these features calls for investigation for malignancy and other causes, not a diagnosis of globus.

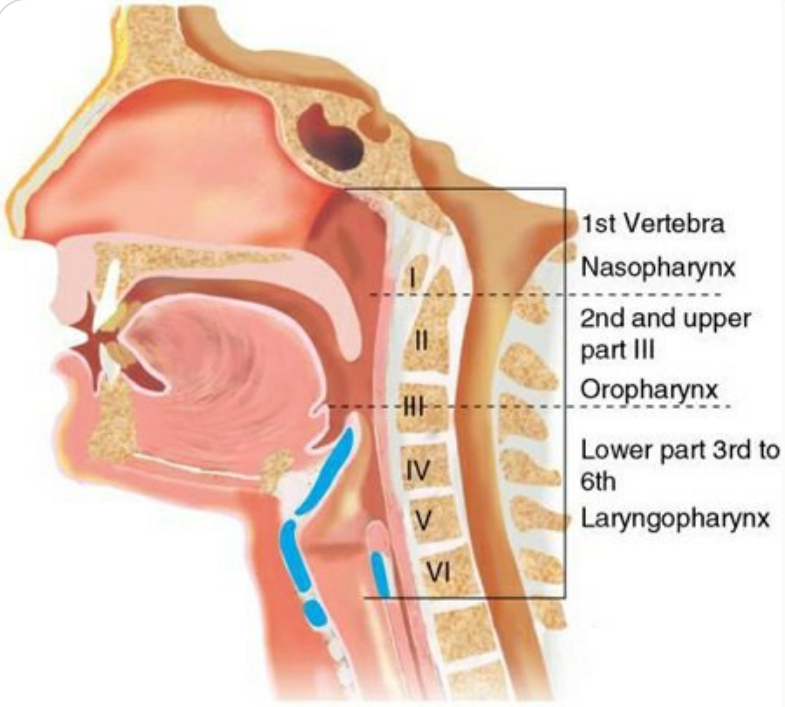


FIGURE 1 Divisions of the pharynx and their vertebral levels.
Dhingra, fig. 47.3

2 CHAPTER TWO Assessment

Examine, then stop.

2.1 Examination

Examine the mouth and oropharynx, palpate the tongue base and tonsils, examine the neck and thyroid, and perform a **flexible endoscopy** of the nasal cavity, nasopharynx, oropharynx, hypopharynx and larynx, including pooling in the piriform fossae and the postcricoid area. Look for signs associated with reflux (interarytenoid thickening, oedema), but recognise that these are common and non-specific.

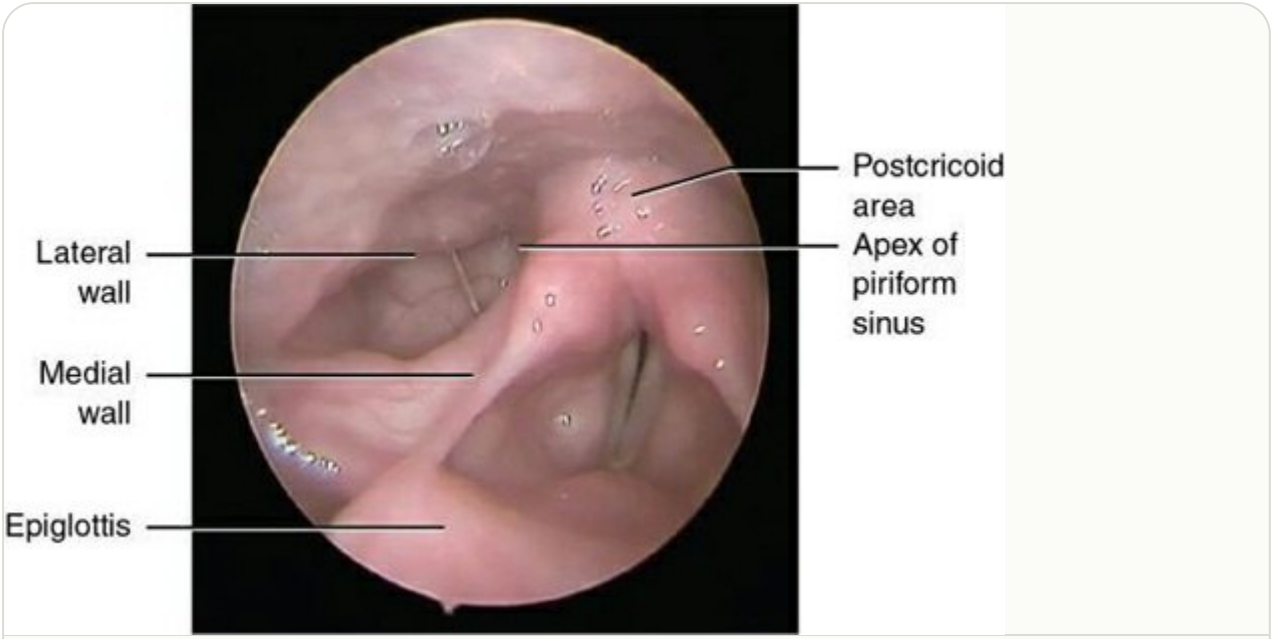


FIGURE 2 The piriform fossa, where saliva pools in obstruction and where hypopharyngeal cancer hides.
Dhingra, fig. 47.8

2.2 Investigation

In typical globus with a normal examination, **no further investigation is needed**. A routine barium swallow adds little and should not be requested just to reassure. Investigations are reserved for atypical features: an upper GI endoscopy or barium swallow for dysphagia, an ultrasound for a thyroid swelling, and imaging or an examination under anaesthesia when malignancy is suspected.

3 CHAPTER THREE Management

Explanation is treatment.

3.1 For most patients

A clear explanation that the sensation is common, real, benign and not a cancer, after a full examination, is the most effective treatment, and many patients improve with reassurance alone. Reduce throat clearing (sipping water or swallowing instead), stop smoking and keep well hydrated.

Speech and language therapy helps patients with throat clearing, muscle tension and voice symptoms. A **trial of reflux treatment** (diet and lifestyle changes, alginates after meals and at bedtime, and a proton pump inhibitor for patients with typical reflux symptoms) is reasonable, though its effect in isolated globus is unproven. For persistent, troublesome symptoms, low-dose amitriptyline and help with anxiety may be tried.

3.2 Review and discharge

Patients with typical globus and a normal examination can be discharged with advice to return if new features appear: pain, dysphagia, weight loss, hoarseness or a lump.

A LUMP IN THE THROAT

1 **Typical?**

→ midline, between meals, eased by eating, no pain or dysphagia: step 2

→ pain, dysphagia, weight loss, hoarseness, unilateral or neck lump: investigate for malignancy — endoscopy and imaging

2 **Full examination**

→ flexible endoscopy, neck and thyroid

→ **abnormal**

investigate the finding

→ **normal**
step 3

3 **Treat**

→ explanation and reassurance

→ reduce throat clearing, hydrate, stop smoking

→ speech therapy

→ trial of reflux treatment if reflux features

→ discharge with safety-net advice

4 CHAPTER FOUR For the examination

What the examiner wants to hear.

4.1 Viva essentials

WHAT THE EXAMINER IS LOOKING FOR

- **The typical features** of globus and why eating eases it.
- **The red flags** that exclude a diagnosis of globus.
- **A thorough examination,** including flexible endoscopy.
- **Why routine barium swallow is not needed.**
- **Explanation, speech therapy and a reflux trial.**

4.2 Common pitfalls

COMMON PITFALLS

- Labelling a unilateral or painful throat symptom as globus.

- Investigating typical globus repeatedly, reinforcing the patient's fear.
- Prescribing proton pump inhibitors indefinitely without review.

Written for this app from Oxford Textbook of Otolaryngology ch. 48, Oxford Specialist Handbook ch. 3, Oxford Handbook of ENT ch. 3, Dhingra ch. 63. Figures reproduced from the books for private study. Study reference only: follow local protocols and senior advice for patients.

IMAGING

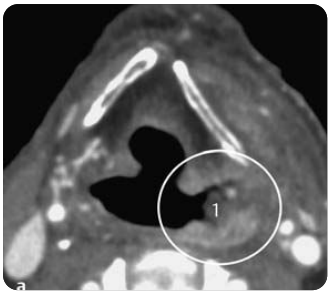
On imaging

The imaging of this topic: key findings, differentials and representative scans.

LARYNX & TRACHEA · TUMOURS

Supraglottic and Pharyngeal Tumors

- Staging is based on enlarged or pathologic-appearing lymph nodes, bilaterality, and overgrowth of the tumor on the contralateral side.
- Lymphomas (non-Hodgkin) initially might present as unilateral lymphadenopathy and ulcerative lesions.
- Evaluation of systemic disease and lymph nodes in other regions as well as secondary tumors in hypopharynx and larynx.
- Keep in mind that micrometastases may be radiologically occult.



CT with intravenous contrast enhancement, axial. Axial slice at the level of the thyroid cartilage shows a soft-tissue mass (outlined as…

THYROID & PARATHYROID · INFECTION & INFLAMMATION

Hypertrophy of the Thyroid Gland, Thyroid Goiter

IMAGING FINDINGS

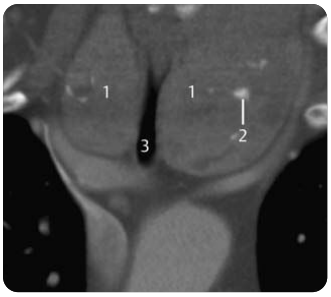
- Enlarged, heterogeneous gland with nodules, cysts and coarse calcification
- Tracheal deviation and narrowing
- Retrosternal extension

TOP DIFFERENTIALS

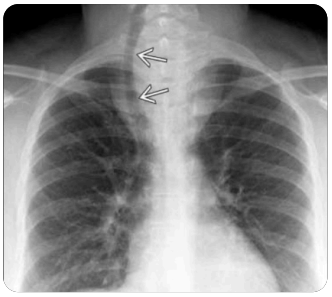
- Thyroid adenoma
- Differentiated or anaplastic carcinoma
- Lymphoma (rapid growth, Hashimoto)

CHECKLIST

- Lowest extent relative to the aortic arch and brachiocephalic vein
- Tracheal calibre for anaesthesia



CT with intravenous contrast enhancement, coronal. This slice at the level of the lower neck/upper thoracic cavity, shows enlarged thyroi…



Posteroanterior chest radiograph demonstrates marked displacement of cervical and thoracic trachea by a large, left-sided neck & superior…

Key facts after Koch, Hamilton, Hudgins and Harnsberger, Diagnostic Imaging: Head and Neck, 3rd ed., p. 226.